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See Online for appendix



Problems in defining medicalised FGM and proposed solutions

WHO defines female genital mutilation (FGM) as “all procedures that involve partial or total removal of the external female genitalia, or other injury to the female genital organs for non-medical reasons” and medicalised FGM as “situations in which FGM is practiced by any category of health-care provider, whether in a public or private clinic, at home, or elsewhere”.¹ Female genital cosmetic surgery (FGCS)—including the most common form, labiaplasty—similarly involves partial or total removal of the external female genitalia for non-medical (ie, aesthetic) reasons and is practiced by a health-care provider, usually a medical doctor, in a public or private clinic.

In other words, the distinction between FGCS and medicalised FGM is unclear.² In practice, the term medicalised FGM is applied exclusively to practices primarily performed by people of colour in countries in the Global South (such as in parts of

Africa, the Middle East, and south and southeast Asia), and excludes anatomically comparable practices in countries in the Global North (such as in Europe or North America),² raising concerns about discrimination and double standards.

It will be objected that, in practice, medicalised FGM is performed on underage girls³ and that FGCS is agreed to by consenting adults. We agree this is a crucial moral distinction, and we include it in our proposed ethical framework below. However, WHO's definition of FGM, whether medicalised or unmedicalised, makes no mention of consent, and so conflates voluntary procedures chosen by adults with those forced on children (appendix). Moreover, in the USA, up to 20% of labiaplasties (ie, FGCS) between 2016 and 2019 were performed not on adults, but on legal minors.⁴ Medically unnecessary surgeries to normalise the genitals of babies with intersex traits, including clitoral-reduction procedures, are also commonly performed in Global North countries, entirely without the consent of the individual.⁵ Finally, virtually all groups that practise vulval cutting as part of a rite of passage (ie, FGM), also practise cutting of the penis or foreskin of boys of a similar age—in both cases often constituting the very ritual that confers adult status on the initiate.⁶ If age or consent matters, it should matter for everyone.

Another objection holds that FGCS, unlike FGM, can be conducted for medical reasons—for example, to relieve psychological distress. Problems with this objection, and other shortcomings of the WHO definition, are outlined in the appendix. To address these issues, we propose a clear ethical standard that does not discriminate based on cultural origin, sex, or gender.⁵ We define unethical genital modification as all procedures that involve partial or total removal of a person's genitalia, or other injury to a person's genital

organs, that are medically unnecessary and done without the informed consent of the affected individual (the terms medically unnecessary and informed consent are defined and elaborated upon in the appendix).

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Nakalanga syndrome and hypopituitarism

In relation to the Review on hypopituitarism by Maria Fleseriu and colleagues,¹ we would like to raise awareness about Nakalanga syndrome, a poorly understood condition most likely caused by hypopituitarism. This syndrome was first described in the late 1950s among the inhabitants of Mabira Forest, an onchocerciasis-endemic area in south-eastern Uganda.² This syndrome is characterised by emaciation, stunted

growth, delayed sexual development, mental impairment, facial dysmorphism, and thoracic and spinal abnormalities.² Since its initial description, it has been reported in numerous other onchocerciasis-endemic areas with high *Onchocerca volvulus* transmission and a high epilepsy prevalence, including nodding syndrome.³

However, research on its pathophysiology remains scarce. Only one post-mortem exam has been conducted, in 1956, on a man aged 30 years with Nakalanga syndrome who lived in the Mabira Forest, revealing thyroid hypofunction and an absence of germ cells and male sex hormones in the testes. His pituitary gland showed few basophils but a normal number of acidophils.⁴ Endocrinological investigations revealed low levels of insulin-like growth factor (IGF-1), IGF binding protein 3 (IGBP-3), or both in two individuals with nodding syndrome and moderate to severe stunting, further suggesting involvement of the pituitary axis.⁵

Nakalanga syndrome ceases to appear in onchocerciasis-endemic areas once onchocerciasis is eliminated, suggesting that an *O. volvulus* infection plays a direct or indirect pathogenic role.³ However, to understand the pathophysiological mechanism of Nakalanga syndrome, additional hormonal studies including pituitary stimulation tests need to be conducted. This effort will require strengthening the research capacity in remote onchocerciasis-endemic areas.

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Concerns regarding Gaza mortality estimates

We respond to a Correspondence by Rasha Khatib and colleagues.¹

Of course, numbers count. Each and every death matters profoundly. However, the culpability and responsibility for each and every death also matter. That is why projections of fatalities must be seriously and soundly based, scrutinised, and not agenda-driven, and most importantly, should thoughtfully endeavour to assign responsibility and accountability if they are actually meant to minimise and even arrest continuing death, destruction, and despair. Providing model-based projections, devoid of appropriation of Hamas' accountability for Gazan civilians, reveals not scholarship, but an agenda that aligns well with that of Israel's most bitter enemies bent on its destruction. Failure to assign responsibility impedes the cessation of hostilities in Gaza, which is necessary to mitigate further deaths, both direct and indirect, and achieve recovery. There will be no hope for such recovery so long as Hamas continues to hold the hostages it captured in its premeditated, vicious, and murderous attack perpetrated on Israeli civilians on Oct 7, 2023^{2,3} (not mentioned by Khatib and colleagues as the trigger for the unfolding tragedy). There will be no hope for such recovery so long as Hamas is able to resume its strategy of embedding military attack capability within the civilian population and facilities while violently compelling Israel to defend itself against Hamas' attacks.^{4–7}

Khatib and colleagues argue that “Documenting the true scale is crucial for ensuring historical accountability and acknowledging the full cost of the war.”¹

Truth is elusive in the midst of war, and documenting is difficult, but necessary. Even if we accept the unverified numbers of direct civilian deaths provided by the Gaza Health Ministry, Khatib and colleagues deliver forecasts of indirect deaths using multipliers based on unsubstantiated assumptions, and then add some more tens of thousands to achieve a thought massacre, undermining credibility. Rather, the emphasis should be on how to minimise this multiplier by preventing indirect deaths. We believe that such mitigation can be achieved when Hamas lays down arms, releases Israeli hostages, stops attacking humanitarian relief, and reverts medical facilities to the provision of medical care rather than to combat command, and that such actions would allow the Israel Defense Forces to cease their combat operations, which are carried out in defence of the people of Israel from relentless Hamas attacks, and would allow relief to actually be provided to those in need, unfettered by Hamas malevolence. This point is entirely missed by Khatib and colleagues' provision of a projected number of deaths that has now been adopted as inevitable.

In now-deleted posts on X, subsequent to publication of the Correspondence, one of its authors seemed to acknowledge that this number is “purely illustrative”.⁸ This “purely illustrative” number, weaponised against Israel, serves as a dangerously irresponsible provocation, and is now nefariously held up as prima facie evidence of a deliberate massacre of an incomprehensible number of non-combatants by Israel.^{9,10} Did the authors not realise how such blood libels are incited precisely by an authoritative scholar's “purely illustrative” example that was published as part of a



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